



Now let me tell you in my own words: narratives of acute and chronic low back pain

Kerryellen Vroman, Rebecca Warner & Kerry Chamberlain

To cite this article: Kerryellen Vroman, Rebecca Warner & Kerry Chamberlain (2009) Now let me tell you in my own words: narratives of acute and chronic low back pain, *Disability and Rehabilitation*, 31:12, 976-987, DOI: [10.1080/09638280802378017](https://doi.org/10.1080/09638280802378017)

To link to this article: <https://doi.org/10.1080/09638280802378017>



Published online: 21 Jul 2009.



Submit your article to this journal [↗](#)



Article views: 1279



View related articles [↗](#)



Citing articles: 10 View citing articles [↗](#)

RESEARCH PAPER

Now let me tell you in my own words: narratives of acute and chronic low back pain

KERRYELLEN VROMAN¹, REBECCA WARNER¹ & KERRY CHAMBERLAIN²

¹College of Health and Human Services, University of New Hampshire, Durham, NH, USA, and ²School of Psychology, Massey University, Albany, New Zealand

Accepted July 2008

Abstract

Purpose. In the past, qualitative studies have focused on chronic low back pain (LBP), yet 90% of LBP is acute and episodic. The purpose of this study was to examine the broader experience (acute as well chronic) of LBP in the community.

Methods. This study was part of a Personal Project Analysis of adaptation to LBP. Participants answered an open-ended question that invited them to tell researchers about their LBP. The narratives were analysed using thematic content analysis and structure was analysed using the narrative types described in 'The Wounded Storyteller' (Frank A. The Wounded Story Teller: Body, Illness and Ethics. Chicago: The University of Chicago Press; 1995).

Findings. Two themes were identified: 1) the challenges to the authenticity of LBP and 2) the consequences of living with LBP, which had two threads: the disruption of life due to physical limitations, and the emotional distress incurred. The narratives were solely told as chaos narratives.

Conclusions. The emotional and behavioural responses of individuals with LBP and the tensions that exist between the individual and others, especially healthcare providers, is explained in the context of LBP as a moral event. Understanding the narratives as chaos narratives provides insight into ways to improve the quality of the interactions between patients and health care providers.

Keywords: *Low back pain, thematic analysis, chaos narratives*

Introduction

In this study, we explore the themes of the content and the structure of acute and chronic low back pain (LBP) narrative. People describe their LBP, a largely private invisible experience, subjectively. They describe pain, difficulty doing activities and their emotional distress emanating from the pain. However, their voice is often undervalued and unheard, since it is the observable objective signs that are valued as reliable indicators of LBP [1]. People seeking treatment for LBP want investigative medical procedures to provide tangible evidence of structural or pathological abnormalities for their symptoms [2]. Tests such as X-rays that the physician orders implicitly reinforce lay assumptions that evidence of pathology will validate their self-reported pain [3]. Such emphasis on the objective signs of LBP is

problematic, since approximately 80–90% of LBP is diagnosed as non-specific or idiopathic because there is no pathology (e.g. abnormal X-ray findings, neurological impairment) [4]. Even when pathology is present, the severity of LBP symptoms (e.g. pain intensity or physical limitations) does not reliably correlate with the extent of structural abnormality or pathology. The ambiguous clinical presentation of LBP and the undervaluing of self-reported symptoms accentuate the lack of authenticity for people's LBP and increases their vulnerability to the stigma associated with pain of unknown cause [2].

In the past, narratives have been used effectively to understand how people with chronic LBP experience, cope with, and navigate their sense of self. A small number of studies have explored the personal experience of LBP and its effect on self-identity

[5–7]. The majority of studies have focused on LBP patients' experience in the healthcare system [2,3,8,9] with an emphasis on the unsatisfactory and sometimes contentious relationships between patients and their physician(s) [10–12]. However, our understanding of LBP is incomplete, because narratives of acute LBP have been largely neglected. Fifteen to 20% of the population has LBP at any given time, but only 10% of this LBP becomes chronic. Consequently, 90% of LBP experienced annually has been omitted from existing research.

Story telling... How we and others understand illness

When study participants were asked, 'What would you like to tell the researchers about your LBP', they responded with stories. In this study, the words 'story' and 'narrative' are used interchangeably. Typically, stories are about past and present events and they have a plot and a beginning, middle and an (anticipated) end [13]. There is a restorative value in the ownership and telling of one's illness story [14]. In the telling of an illness narrative, the meaning of the illness experience evolves, much like the illness itself changes over time. The emphasis of an illness narrative can shift, and its content and structure may be modified [13] and an individual finds meaning as well as recognition of his/her experience. Finding making sense of the meaning of an illness in the context of one's life is important, because illness is not only about the body; illness also threatens identity [15]. Telling one's story helps a person begin to discover him/her self in the context of changing body and a life story that must now include an illness.

Analysis

Thematic content analysis, germane to qualitative methodologies (e.g. ground theory, interpretative phenomenological analysis), is increasingly being regarded as a method in its own right [16,17]. Braun and Clarke [16] argue that thematic analysis provides a theoretical freedom, which achieves a 'rich and detailed, yet complex account of data' (p. 78). Because themes may represent both patterns of response and meanings across data sets as well as within specific data (i.e. an individual interview), thematic analysis was particularly applicable to the current study. However, Murray [14] reminds us that there are levels of analysis in qualitative health research and that analysing the structural level analysis provides an understanding of the intersection of peoples' experiences, meaning and culture [15]. This is important given the salience of the socio-economic and medical context to the experience of LBP.

Understanding the structure of low back pain narratives

Illness stories, like other traditions of story telling, have general storylines that are revealed in the underlying plot and tensions of a narrative. A storyline or *narrative type* represents the main points of a plot that a person wishes to convey [15]. Although a story may not conform exclusively to a specific type of narrative, one type will usually predominate and guide a narrative. In the *Wounded Storyteller*, Frank [15] proposed three types of storyline structure which guide illness narratives: restitution, chaos and quest. Restitution narratives, the most common, are about transitioning from health to sickness and the return to health (i.e. diagnosis, treatment and cure). The plot explores the experience and meaning of investigations and treatments that identify and defeat the 'enemy' (disease/illness). Medical professionals embrace these narratives, because recovery is often attributed to successful medical intervention in which they are central figures. We are drawn to, and to some degree, expect people to tell restitution narratives about their illness. It is part of the sick-role; they receive care, are exempt from their usual responsibilities and in return are expected to 'recover'. Lance Armstrong's remission from testicular cancer and triumphant return to winning the Tour de France is a quintessential restitution narrative.

The second type of narrative, the chaos narrative, is the antithesis of the restitution narrative. These narratives lack order and are about struggling with vulnerability, and the disruptions caused by illness [18]. Although restitution narratives have a future and are imbued with hope, chaos narratives convey the inescapability of illness, pain, suffering and possible death. Hence, these narratives evoke fear. Unlike restitution narratives, it is difficult to listen to chaos narratives and consequently people avoid them. This denies the storyteller the opportunity to tell his/or story and work towards a sense of meaning. It also denies the person recognition for his/her suffering.

The third narrative type, the quest narrative, is philosophical and attributes illness to fate or destiny. The plot of the quest narrative is about achieving meaning, and finding a sense of purpose from an illness experience. In classic Judeo-Christian tradition, a person is tested, he/she suffers and through the trials and tribulations overcomes challenges to emerge a better person or have an epiphany. The outcome is often self-awareness, personal growth, meaning or the impetus for social action [15]. Recovery is not an essential element of a quest narrative. Mitch Albom [19] provides a popular example of the quest narrative in 'Tuesdays with Morrie.'

Current study

Mindful of representing the personal experiences of LBP, accounts of participants' acute or chronic LBP were included in the data collected in a Personal Project Analysis (PPA) [20] study of adaptation to LBP in everyday life using a representative sample of community dwelling people with LBP who sought treatment. In keeping with the methodology of PPA, all the current personal projects individuals were engaged in or planning were elicited. Then their five most important projects were appraised by them on 26 items. These scores were collapsed across the five projects to ascertain participants' disposition toward their everyday activities as predictors of well-being and function [20]. The results of this part of the study are reported in Vroman, Chamberlain and Warner (submitted for review). The current article focuses on the qualitative data provided by participants in this PPA study. In presenting the analysis of this data the authors honour the participants' trust that in sharing their stories, the researchers might listen to and appreciate their experiences of LBP.

Procedure

The study was approved by Institutional Review Board for the Protections of Human Subjects (IRB) of the principal researcher's affiliated University and from the Regional Hospital Board IRB that had oversight for several of the data collection sites. Protocols for patients' anonymity were in accordance with the United States Health Insurance Portability and Accountability Act of 1996.

In the PPA study, the lengthy questionnaire ended with an open-ended question: 'Now we would like to give you a chance to tell us about your experience of back pain in your own words. If you could talk to the researchers, what would you like them to know about LBP?' The original intent of this question was to substantiate the content validity of the PPA data, but what emerged were rich descriptions of participants' experiences of LBP. Ninety-three percent, 133 of the total 143 participants in the study answered the open-ended question. It was not enough to complete the lengthy questionnaire, participants' needed to 'tell their story in their own words.' and personalise the information they had provided. As one participant wrote, 'You have probably heard all you want about the pain, but...'. This participant had probably spent 60 minutes completing multiple tasks in the questionnaire about his back pain, yet he, like others, needed to tell his story. The stories varied from a few sentences to several pages.

Participants

All participants in the study had a diagnosis of LBP (musculoskeletal pain below the 12th thoracic vertebra and above the gluteal fold). They were recruited from healthcare facilities (e.g. physical therapy, occupational health or ambulatory medical centres, chiropractic or osteopathic clinics) in the Northeast New England States. Forty-two percent ($n = 56$) of participants who responded to the question had acute LBP (pain for 3 months or less) and 58% ($n = 77$) had chronic LBP (persistent pain longer than 3 months). Fifty percent of the episodes were a recurrence of LBP. People with LBP attributed to malignancies, chronic inflammatory disorders (e.g. rheumatoid arthritis), pregnancy or primary infections and people who were recently post-surgery were not recruited.

Participants were between 19 and 83 years old (median age was 43 years, $SD = 15.51$) and 49% were between 40 and 60 years old, which is peak prevalence for LBP. Consistent with regional demographics, 96.9% of participants identified themselves as White (non-Hispanic). The majority of the participants (62.4%, $n = 83$) were employed full-time, 12% ($n = 16$) were employed part-time and 3% ($n = 4$) had temporary employment. Eighteen percent ($n = 23$) were either retired or were students and the remaining 5% ($n = 7$) were receiving disability or worker's compensation. Among the participants who were employed, 17% ($n = 17$) were on sick leave from their jobs because of their back pain. This pattern reflects current medical protocol of continued employment (on modified duties if necessary) for people with LBP. All educational levels were represented, but the number of participants with college degrees was high (54%, $n = 72$), largely because several recruitment sites were affiliated with universities.

The participants with acute and chronic LBP did not differ significantly on psychometric measures such as function, quality of life, depression and pain severity [20]. Furthermore, the themes of the acute LBP narratives were similar to themes of the chronic LBP narratives as well as those reported in previous studies that were exclusively chronic LBP [3,7,10,20].

Analysis

The investigators sought to identify and describe the themes within the participants' narratives and followed the standard peer-review process for thematic analysis [22]. The narratives were independently coded for meaningful units by the first author whose background is in mixed methodologies, and an academic colleague, a qualitative researcher. Both coders' shared a professional background in occupational therapy and thus have a common paradigm of

adaptation, the major focus of the overall study. The units of data were labelled by the coders, who then identified salient commonalities, patterns and differences, this dynamic process generating a thematic map that assigned categories (themes). The narratives were reviewed using these themes. In a final step of the process, the second and third authors were involved in the process. There was agreement that the themes that emerged were those of authenticity and legitimacy of pain, and the consequences of having LBP, which included two sub-themes of the disruption of everyday activities due to physical limitation and the emotional consequences of the disruption due to pain. Both sub-themes were present in many of the narratives, though there tended to be an emphasis on either the physical or emotional consequences.

In the discussions among the researchers that developed the themes, we explored whether the pattern of describing symptoms was a theme of the narratives. However, the meaning of initially providing the descriptions and the affective nature of the descriptions lead the researchers to agree that it was participants' need to legitimise their condition (authenticity of their LBP) that underscored this narrative content and pattern.

'Let me tell you about my low back pain' – themes from the narratives

Familiarity can create a complacency that interferes with our ability to listen critically. LBP is so common, it is easy to assume we know the whole story; after all, it is highly likely we have had LBP at some point. Therefore, we begin this exploration of LBP by visiting two stories, one about acute LBP and one about chronic LBP.

Story 1: Participant #4, 43 years, female, acute low back pain

My pain began in my tailbone 2 years ago and to date there has been no definitive diagnosis. I have tried self-care; chiropractic care; PT; medications; ice and heat; cortisone shot; diagnostic tests i.e. CT scan, MRI, sigmoidoscopy, laparoscope, acupuncture. It cleared for a period of about 9 months but has now returned. I am concerned that my chosen career of occupational therapy will aggravate my pain and this is an emotional dilemma. Not knowing the reason for my pain is extremely stressful and frustrating. This makes me angry and I feel it alters my personality to a degree.

Story 2: Participant #52, 80 years, female, chronic low back pain

My first episode with back pain came like a malignant lightning bolt after my daughter was born. An osteopath set things right, but since then I've had recurrences of

back pain until now I am never without pain. Ten minutes of work, whether as benign as standing to wash dishes, it results in so much pain I must lie down. I enjoy gardening, keeping the lawns mowed and many other things, but I always pay a penalty in pain and 'time-outs' to gain relief so that I can fix a meal or do other things. To compensate for things that cause pain, I've resumed work on an elaborate Georgian-style dollhouse, which is presenting me with challenges of skill, imagination, and artistry. I use the dollhouse project as an incentive: 'No work on the house until you've done the vacuuming'. (A chore I never seem able to complete because the activity brings on much pain. The carrot-on-a-stick method works for me and makes my dollhouse project even more precious.

LBP is a collection of symptoms desperately searching for corroborating clinical signs [3,23], which is also true of participants' LBP in the current study. Participants identified the cause and described their LBP to corroborate their symptoms and justify their condition. Above, one woman still attributes her LBP to childbirth that occurred 50 years earlier. The other woman, not having a specific diagnosis, goes to considerable length to explain and assert the seriousness of her condition by listing numerous diagnostic tests. She particularly illustrates the importance participants gave to having a diagnosis or tangible cause. Like the women's stories, other participants also described an injury or a diagnosis (or lack of diagnosis). They referred to pathology, structural abnormality, trauma, or offered a self-diagnosis using lay or quasi-medical language to legitimise their conditions. For example:

It first developed lifting a 600 lb. Stove. My back 'ripped' while building my house 16 years ago. It comes every 3–6 months if I lift or strain, goes away in 2–3 days, spasms last 1–2 days. Recent injury lasted 6 weeks—therapist said it was a torn muscle in back area, not disc. (P#11, male, 52 years, acute LBP)

A therapeutic function of telling an illness narrative is to remove guilt from the narrator and balance the relative responsibility for the illness [14]. The need to assert that their LBP is not their fault was particularly salient in participants' narratives. In part, participants achieved this by making their pain a 'real' bodily disorder. Hence, cause, based on objective evidence of injury or pathology within their physical body, provided participants assurances that their condition would be regarded as legitimate. This process, often a prologue to their descriptions of lives disrupted by LBP, losses, and suffering, seemed to be a pre-emptive strategy to make them less vulnerable to other people's sanctions. Paradoxically, it accentuated their vulnerability to perceptions of

people and reflected a heightened need for recognition of their suffering.

The extent to which an illness experience is accepted by others is the degree to which it is a sickness with social meaning [24]. Participants attempted to make their symptoms real using medical terms and descriptions of causality, as if establishing the credibility of their LBP was a priority. Campbell and Muncer [25] suggest lay theories of cause overlap with medical accounts of physical symptoms and serve to create order from the fragmentation illnesses such as LBP produce. However, the similarity of participants' descriptions seemed to serve an additional purpose. It was as if participants were conforming to a pre-determined script for telling other people about LBP as if they implicitly understood a social pressure to present their pain as credible (i.e. an illness with a discernible cause).

'Other people do not understand ...' Participant pursuit of authenticity

A theme across the narratives was that other people failed to appreciate their suffering and because they appeared outwardly normal, did to take their pain [i.e. them] seriously. Having the authenticity of their pain questioned augmented their emotional distress. They felt misunderstood.

Back pain is invisible, therefore hard to describe, it is debilitating. Back pain truly hurts. (P#1, female, 32 years, acute LBP)

It is something that has to be experienced to be truly understood. (P#18, male, 43 years, acute LBP)

Chronic pain is very difficult to live with. It greatly affects a person's emotional well being. Most people require a great length of time to learn how to deal with the pain. This usually involves putting tremendous strain on family and friends. If they are patient AND supportive, it is a huge factor in learning how to live with your pain. SOMEONE WHO HAS NOT EXPERIENCED SEVERE AND LONG TERM BACK PAIN HAS NO IDEA HOW BAD IT CAN BE AND OFTEN CAN GIVE BAD ADVICE!!!! (P#16, 49 years, male, chronic LBP)

There was a desire for empathy and participants felt they were denied a level of understanding that they believed they deserved.

My back pain has not decreased, but my employment has not been supportive with this so I have had to live with the pain (P#03, male, acute LBP)

Sometimes others are less patient with the disability because, except for the reported pain, sufferers look and

function normally. LBP doesn't keep my brain from working; it just creates a distraction that is not apparent. (P#107, female, 53 years, acute LBP)

... Because I can't do activities or tours, my husband gets disgusted because I can't keep up with him ... (P#25, female, 60 years, chronic LBP)

I hate having back pain and feel that others do not understand when it limits my activities. I get depressed and discouraged. (P#142, 44 years, acute LBP)

An element of others questioning the authenticity of their LBP was articulated in the distress and frustration experienced in interactions with medical professionals. Participants felt they did not always receive the treatment they desired or deserved and were distressed by vague or absent diagnoses. This was associated with a lack of confidence in the ability of medical professionals to understand and effectively treat their condition.

I become extremely stressed when dealing with insurance companies and some doctors who don't really care about your pain. People treat you very differently when you are disabled and have to use a cane or wheelchair. I'm not hopeless. (P#19, female, 30 year, chronic LBP)

Doctors don't seem to be able to fix the pain. (P#40, male, chronic LBP)

My back pain gets so bad I want to cry ... It has stopped me from doing a lot of things in my life. I do not think it is right for me to have to go to several different doctors and keep repeating it every month, every time having to get a doctors' ok for refill on my pain medication. It hurts so bad I feel like taking a knife and cutting it out. (P#50 42 years, female)

Lack of authenticity was interwoven with unmet needs and disillusionment. The descriptions of interacting with the medical providers expressed pessimism, scepticism and low expectations of receiving effective treatment. Although participants reported they often obtained relief from symptoms through therapy (Chiropractic, Physical, Osteopathic Manipulation), the tenor of those comments were that those interventions managed their pain, but they did not alleviate their expectations and desire for medical resolution (i.e. cure).

The consequences of low back pain

The consequences of LBP had two sub-themes: 1) the disruption of everyday activities and 2) the emotional cost and distress of LBP. The narratives of two women illustrate both of these themes. The first woman focused on the emotional consequences and her concerns for her job. The second woman

described the effect of LBP on her activities and the tenuous balance she negotiated to keep her pain tolerable, yet be able to do the activities she wanted to do.

The consequences of LBP: Disruption of everyday activities

The most common reason people seek treatment for LBP is the physical inability to engage in everyday activities [2,8]. This was a powerful sub-theme of the consequences of having LBP. The suffering and disability of their LBP, which disrupted their lives, exemplified the experience of LBP. Disruption to their lives included simple tasks, such as dressing, driving and housekeeping as well as difficulties at work and engaging in sport activities. The excerpts below portray LBP, an omnipresent entity in the participants' lives, as unpredictable and exhausting. Although comments about the unpredictability of LBP were more evident in the acute narratives, the frustration and inability to carry out everyday activities was universal.

Low back pain can turn your world upside down. Life goes on hold while you deal with the symptoms. The Health Care providers who have helped me the most have demanded I become actively involved in my own treatment. I have lower back pain, it's not going to be "cured," but I can control its effect on my life to some degree. (P#10, female, 31 years, acute LBP)

Low back pain is always ready to reappear. It can be immobilizing at times and then seem to disappear and lull one into a false sense of security. Acute attacks are usually without warning and can be very disruptive. Recovery time is prolonged and can be very frustrating because the pain can remain as a chronic irritant. Chronic pain is very tiring. It inhibits activity and interaction and saps strength. (P#107, female, 53 years, acute LBP)

I was a high functioning woman who had 2 part-time jobs, exercised 1–2 hours 5 times a week and maintained a home. I am now an unable to do any of that. I have so much pain I am unable to work my job as a personal support worker and Phlebotomist. I am unable to bathe or dress myself. I now depend on my husband to do these things for me. I also have sharp stabbing pain that radiates down the back of both of my legs to my heels. I also experience numbness and pins and needles in my legs from waist to my toes. I've also experienced incontinence of stool and urine when I have this numbness. Narcotic medications have not been successful in preventing m pain. Acupuncture provides me with temporary relief. (P#92 male, 30 years, chronic LBP)

Participants described two approaches in dealing with their LBP that were similar to patterns previously identified in chronic LBP patients [9,26]. As illustrated in the narrative of participant

#107, one way was proactively to manage her symptoms. Statements of defiance and a determination to manage LBP characterised this type of approach. These individuals described actively avoiding the sick or victim role. They attempted to continue daily routines, stoically refusing to permit their pain to redefine their perception of themselves and their lives. They voiced resolve to confront their pain, while also acknowledging the challenges it presented. Pain was an entity that they had to deal with, yet they also made a clear distinction between who they were (self) and the pain. Their narrative described a battle waged against an unwelcome intruder and an underlying commitment not to let pain interfere with their activities, and therefore not affect who they were as an individual.

I fight the pain even when it wears at me and it makes me tired, I won't let it win." (P#91, male, 46 years, acute LBP)

I know it is not life threatening. I know how to get rid of severe pain. I refuse to let my back pain interfere with my life. (P#34, female, chronic LBP)

The alternative pattern described by participants was to acquiesce to their LBP. In these narratives, participants described how their pain prevented them from doing activities and living a life that they envisioned was only possible without LBP. These narratives commonly described the loss of enjoyable activities and of self.

My biggest complaint is that I've had to give up many things that I used to enjoy and burn off stress with-skiing, backpacking, skating, aerobic dance, golf, etc. Additionally, I've had to change the way I dress as I can no longer wear high heels which makes me feel frumpy. (P#78, female, 45 years, chronic LBP)

I am aware of my back in almost everything I do and in all waking hours. It is one of the constant forces that dominants my life. It does keep me from doing things I want to do (P#51, male, 47 years, acute LBP)

These descriptions revealed how demoralised the participants were. Pain was portrayed as an entity that had control of them and their lives. These participants perceived they were victims of their pain and events were beyond their control. They relinquished personal agency to their LBP. It was their adversary, and their pain defined their self-identity. In these descriptions, pain had agency.

Sometimes I feel it has taken over my life. (P#79, female, chronic LBP)

Not only does pain affect my life, it affects those around me. I have to cancel plans, appointments, not able to

help family or work. It makes me very unreliable and I can't even tell from one day to the next how I'm going to function. It's frustrating and depressing especially when your body won't function the way your mind is telling it. (P#115, 35 years, female, acute LBP)

It is a constant force, which dominates my life. It keeps me from doing things I want to do. (P#51, male, 47 years, acute LBP)

Participants' who described organising their lives around their LBP also revisited fears of re-injury or an exacerbation of their condition. Somatic pre-occupations, pain-related fears, and actively limiting activities shaped these people's identity as one defined by their pain. They described constant vigilance to prevent further episodes.

Sometimes I over protect myself. (P#33, male, acute LBP)

It's surprising how back pain affects everything as the dull ache is with you all day. I found myself limiting myself subconsciously from doing thing I normally would just do to try and not get a flare up of the pain. (P#68, female, 42 years, acute LBP).

The consequences of LBP: Psychological distress and loss of the self:

The second sub-theme was that pertaining to psychological distress. Regardless of whether participants approached their LBP as living their life despite LBP, or living a life organised around LBP, their narratives were similar in the emotional distress they experienced. They shared a common desire to be pain free, hoped for full recovery, felt hopeless, and feared never being able to return to their former (pain-free) self. The loss of meaningful activities was a significant element of this distress.

Back pain is probably the most painful, most frustrating experience I've ever had. It affects so many aspects of my life- mood, activity level, and motivation, social, physical, self-esteem, etc. For me, it's difficult because I am typically a very active and independent individual. I can't drive means I rely on my husband to transport me ... I have felt angry, sad, frustrated and weak the past two weeks at some point or another - not everyday, but more so then if I were feeling pain free and were able to perform my daily tasks independently. I really miss working out, running, and lifting. I fell like I'm losing time every day - which I've worked very hard for years to maintain - I resent my body for failing me, especially when I work so hard to be good to it!! I find it frustrating that it takes so long to feel better, even though I've made progress. I've been more cranky the last couple of days because I'm out of work and I have difficulty sleeping at night. (I'm tired but I don't do anything!) I guess I've always taken my physical health for granted. (P#55, female, 43 years, acute LBP)

Now I feel more vulnerable and weak. I love sports, coaching, and playing. This injury has made it difficult to do both. I have few real pleasures in life, coaching my boys is top of the list. Not being able to do that, as I want, causes me pain emotionally. My kids are growing and I feel like time is running out and I am missing something. (P#90, female, 38 years, chronic LBP)

I would give anything to have my former life back. (P#22, male, 37 years, chronic LBP)

Even though LBP is a non-life threatening self-limiting disorder, these participants' responses are emotionally intense. Participants described how LBP compromised valued and meaningful activities and consequently it undermined their sense of self. Participants' stories illustrated that participation in identity-related activities was a crucial and visible representation of self, and that thus the loss of such activities augmented their distress. Furthermore, disruption of activities threatened social roles (e.g. parent, husband) intimately tied to identity. The two excerpts from the stories of participants #23 and #112 below illustrate the challenge to a sense of self that was associated with losing valued identity-related activities and the loss of place in their social world [3,7].

Participants' descriptions of their loss of the ability to engage in seemingly simple activities revealed a psychological cost of not fulfilling roles that are integral to their socially constructed identity.

I have never had back pain prior to this recent incident. I was surprised at how I didn't feel like myself- no appetite, I am more lethargic ... the thing that bothered me most was not being able to hold my baby daughter, and watching my wife have to handle all the daily chores. (P#23, male, 36 years, acute LBP)

I am unable to play with my children and engage in most activities. I struggle to carry a laundry basket and it leaves me feeling less of a mother/wife. I haven't been able to mop my floor in months. (P#112, female, 25 years, chronic LBP)

This woman connects her ability to do household tasks with her identity as a mother and the loss of her sense of self and because she is unable to engage in these routine activities, it intensifies her emotional distress. Interestingly, the distress of these participants was not associated with the duration of their LBP. The man also expresses his difficulty watching his wife assume tasks that were his responsibilities. Both experienced what Smith and Osborn [7] describes as an 'assault on the self'. When the illness causing the disruption of role defining activities lacks social meaning as a sickness, the individual is not exempt from one's expected

duties without sanction. LBP is often not regarded as legitimate and hence the coping with the disruption is more difficult because there is not allowance made for being sick.

Others did not make a connection between their activities and identity. Instead, they focused their emotions and vividly described feelings such as frustration, anger and depression.

My back is doing better with chiropractic care, but before, I was depressed. I felt like “damaged goods” at 32. I was in pain, a lot of pain everyday. I was cranky and sad. I felt bad for my husband and for my children. They were ‘jipped’ . . . I became a hermit. I gained a lot of weight from being sedentary. Back pain can change a happy, healthy individual into a sad, depressed hermit. (P#103, female, 37 years, acute LBP)

I have to cancel plans, appointments, not able to help family or work. It makes me very unreliable and I can’t even tell from one day to the next how I’m going to function. It’s frustrating and depressing especially when your body won’t function the way your mind is telling it. (P#115, female, 35 years, acute LBP)

The consequences of LBP, the inability to engage everyday activities, the ensuing disruption of a life, the emotional distress, and the struggle for self-agency were captured by this participant; ‘If I had to sum up how back pain affects my life, I’d have to say it affects everything . . . Our back is the core of our bodies, which when unable to function normally or effectively, affects our whole being’.

Narrative types: low back pain as a chaos narrative

The thematic analysis explored the participants’ narratives at a personal level to provide understanding of the lived experience of LBP [14]. The themes were about a neurosensory phenomenon, and participants’ attempts to create meaning while negotiating a disrupted pre-existing life story. Although the inclusion of acute LBP narratives enhanced our understanding of how LBP is experienced, thematic analysis alone could not advance our understanding the experiences of LBP that participants described or their emotional distress. We needed to understand the distress that emanated from the transaction of an individual’s experience (regardless of duration) and other people’s responses that were informed by, but also continue to perpetuate a negative social-construction of LBP.

For this understanding, we explored the structure of the narratives to access meaning not directly evident in the words of narrative, but instead revealed in how a narrative is organised and Frank’s

[18] narrative types were used to interpret the narratives’ structure. Frank [18] wrote:

‘Disability can only increase, pain will never remit, physicians are either unable to understand what is wrong or unable to treat it successfully. Medical problems proliferate into social problems: persistent ill health means job problems, which means loss of income, which leads to inadequate healthcare. The ill person is shuffled between bureaucracies, each claiming that they need something from somewhere else before they can provide benefits. Stress exacerbates medical problems. Family responsibility cannot be fulfilled: social ties are lost, and ties that remain are often more demanding of the ill person than supportive’ (pp. 201–202).

Frank [14] wrote this passage to describe chaos narratives. However, it has an uncanny resemblance to the themes of participants’ narratives. Previous studies that report all three narratives across individuals with a similar condition, however, in this study, the participants’ narratives were chaos narratives. Although, studies have reported that narrative type changes with circumstances and the course of illness; for example, a recurrence of cancer may shift a restitution narrative to a chaos narrative [27–29], the participants’ wrote chaos narratives about their vulnerability, frustrations, lack of control, and the unpredictability and indiscernible causality of LBP. Many of their narratives were devoid of hope and resonated with anger, and blaming. Ever-present were participants’ fear that unwittingly they would cause another episode and worsen their current condition, and many felt that their physicians were unable to understand what was wrong, (i.e. tests don’t reveal a cause for the suffering), and are unable to provide a cure. Participants did not expect their symptoms would remit completely. They conveyed a sense of inevitability that is characteristic of chaos narratives.

As identified in the thematic analysis, both acute and chronic narratives reflected participants’ vulnerability to other people’s scepticism, and at times, they felt useless and judged. Similarly, participants’ narratives included the failures of the healthcare system to meet their needs, conflicts with the insurance companies, and family difficulties. Interwoven with these accounts were participants’ narratives about their psychological pain and emotional difficulties. All of these elements are consistent with chaos narratives. Chaos narratives often describe such tensions between the individual and others e.g. the healthcare system, family, and bureaucracies such health insurance companies [18]. Chaos narratives include how illness leads to social problems. Chronic LBP, in particular, is associated with social problems such as work absenteeism, loss of work, financial strain, and long-term disability as

well as substance abuse and relationship difficulties [30].

The frustration, hopelessness and social problems of chaos narratives were reflected in a lack of effective action that was voiced in LBP narratives. A lack of personal agency was accentuated with participants' use of the passive voice; 'it' when referring to their LBP; 'it frustrates me when it prevents', 'I try not to let it affect my life too much' and 'It will slow me down sometimes and it prevent me from doing activities'. Although they construed their LBP as an entity that was controlling them beyond their ability to effect change, they also owned 'it', as belonging to them as part of them (e.g. 'my LBP'). These narratives reflected the turmoil that is common in chaos narratives. When a listener is hearing a chaos narrative, they feel as if they are being sucked into a mire of unresolved issues and emotional pain that has no obvious resolution. The following narrative exemplifies the chaos narratives of LBP.

Story 3 Participant #110, 47 years, male, chronic low back pain

My back pain is ruining my life. There are times when I feel consumed by the pain. It has robbed me of my right to a better, more enriched lifestyle. I have been forced to quit doing almost everything I enjoy. There are times that I am so depressed that I would just go to sleep and never wake up again. All I hear from doctors is "we don't want to give you pain medications because of dangers of addiction." So, I am forced to suffer through the pain. I feel this is extremely unfair to me. I am told there is nothing medically that can be done for me, so, I guess I am left to suffer. I am trying to file for disability, but I will probably be turned down because I am working to pay bills. I drive a taxi for a living and sitting for 12–15 hours per day and picking up and carrying luggage, groceries etc. only aggravates my condition. I know I need to see some specialist or find a doctor that will sign a statement saying I am physically unable to work my job. Then I could get disability long enough to heal and go to vocational rehab and learn to do something else that doesn't aggravate my condition. Then maybe I would not be so depressed about my situation and I could live a healthier, happier life.

This man describes how LBP affects him in the passive voice and the power he gives *it* is explicit. *It* robs him of his agency, rendering him impotent. The narrative lacks order. The teller jumps into his story, with no clear beginning or end. He voices a lack of autonomy. He is unable to leave his current work and find a more satisfying job. He feels ineffectual in his interaction with the social and health care systems. Bureaucracies thwart his efforts to receive a disability benefit and he feels betrayed and unsupported. At times, his distress is unbearable. He writes, 'I am so depressed that I could go to sleep and never wake up'. He believes he is powerless; 'It

[pain] has robbed me of my right to a better more enriched life style'. His narrative overwhelms the listener, just as Frank [15] says, reading this story is like being 'sucked into the undertow of illness' (p. 115).

Discussion

This study brought together the experiences of acute as well as chronic LBP. Although the thematic analysis of participants' narratives adds to our understanding of LBP in the context of everyday life, it is the analyses of the narratives' structure that provides new insights to the significance of how meaning and culture interact to shape the LBP experience. LBP as an illness narrative is one of chaos and although other studies have not specifically analysed the structure of LBP narratives, their accounts suggest their participants did indeed tell them chaos narrative. In particular, narratives provided by women with chronic pain reflect the elements of chaos narratives [31,32]. These narratives resonate with the emotional battering typically present in chaos narratives [10,12]. By assuming that experiences of LBP are told as chaos narratives, we can begin to interpret dynamics and interface between the individual with LBP and others differently and give further perspective to LBP phenomena.

Though back pain is one of many common conditions, it holds 'a special place in the fabric of morbidity' [33, p. 230]. When explaining common disorders, people seldom feel compelled to provide an external causal event [33]. One does not say to the physician 'I caught a cold while ...' However, more often than not a consultation for LBP starts with a preamble such as, 'I injured my back'. This is then followed by an explanation that offers an external, non-culpable cause e.g. 'lifting boxes at work'. Werner, Isaken and Maulterad [32] write of women describing LBP to their physicians. '[it is] hard work ... to make symptoms socially visible, real, and physical when consulting a doctor' (p. 1037). Women's efforts, they state were a, '... balance not to appear too strong, or too weak, too healthy, or too ill, or to smart, or too disarranged' (p. 1037). Participants in our study also seemed to attempt to provide an implicit socially set script for presenting LBP, as if there was a universal belief (conscious or subconscious) that by getting their presentation of their LBP 'correct,' their pain would be viewed as valid and it would not be seen as a personal fault or weakness. Ironically, this behaviour implied they were expecting their LBP to be regarded spuriously or—even worse—that their LBP would not be seen as a bona fide illness.

Illness experiences that involve shame, perceived responsibility, blaming and stigmatisation can be

viewed as a moral event [32,34]. It may seem odd to construe a musculoskeletal disorder triggered by biomechanical stress and physical exertion as a moral event, but this is precisely how participants experienced LBP. The themes of their narratives reflected their struggle with implied blame, alienation associated with a pain disorder of indiscernible cause and distress. This struggle of being maligned, subjected to pity, and having culpability inferred is internalised as shame that results in a devaluing of the self, all of which challenges their moral being [6,7,35].

Stein [22] proposed there are those 'sick' people, who evoke compassion, and there are those who are 'trolls', who may or may not have a bona fide illness, and are accountable for their disease. The insinuation is that trolls are not sick. Therefore they are not eligible for the social entitlements that are conferred upon the sick such as compassion and release from day-to-day responsibilities until they are well. Hence, they exist in an indeterminate state of being neither well nor sick [24,34]. Because of the discrepancies between symptoms and clinical presentation, as well as the subjectivity of self-reported pain, people with LBP sometimes feel like and are regarded as 'trolls' and as previously stated their stories reflect the elements of chaos narratives.

Instinctively chaos narratives create distance between the storyteller and audience. Unlike restitution narratives that reassure the audience, the LBP narratives described a disturbing lack of control that negated hope. The dissonance of LBP told as chaos narratives explains some of the tension between individuals with LBP and others. Physicians willingly admit they are frustrated and demoralised when treating people with LBP, and have even described it as one of the most unrewarding problems to deal with in clinical medicine [36]. Women with chronic LBP in Jackson's [37] study voiced that they felt doctors ignored, disregarded, and rejected them, and that encounters with doctors undermined their self-esteem. These problematic LBP patient-physician interactions typically have been explained as resulting from the futility felt by both physicians and patients and physicians' sense of impotence when treating disorders of an indiscernible cause for which they cannot provide effective interventions. Because LBP patients tell their illness stories as chaos narratives, we suggest that in part the tensions between LBP patients and physicians arises because physicians' inclination in hearing chaos narratives is to address their discomfort by emotionally distancing themselves from the turmoil of the patient as expressed in the chaos narrative. The consequence the physician does not identify with the patient and therefore cannot offer the empathy or recognition of suffering that the patient with LBP wants and needs. The individual with LBP experiences alienation and

denial of empathetic care that s/he desires, but also as a patient, they are deprived of the therapeutic value of telling his/her illness narrative.

Best practice guidelines of medicine acknowledge the significance of the empathetic patient-centred relationship to health and recovery. However, we seldom discuss how difficult it is to listen to and bear witness to confusion, suffering and occasional accusations of a failure to meet medical needs that are common in chaos narratives. To bridge the gap that exists between patients with LBP and physicians requires understanding the nature of chaos narratives in order to listen responsively to them. To engage with the patient who tells a chaos narrative requires that physicians and other healthcare providers accept people's stories and their need to tell them in an attempt to make sense of their illness and help them navigate the experience [13,18,38]. This means honouring a chaos narrative by listening and not moving quickly to action (e.g. medication, surgical interventions, advice and alleviating physical suffering) in an attempt to quiet the discomfort invoked by the narrative itself.

In the case of LBP, this begins with the clinical definition of LBP. The Forum for Primary Care Research of LBP has already advocated a conceptual shift in recommending that LBP be seen as a functional disturbance; a recurrent, mostly manageable syndrome, rather than a disorder to be treated with the objective being cured [39]. This construct is more congruent with the physical experience of LBP that is grounded in the body and the emotional experience that emanates as much from how LBP is construed in western society as it does from the disruption this disorder inflicts on an individual's life. Most importantly, it acknowledges the complex recurrent nature of LBP, while minimising individual culpability.

Limitations of study

Typically, qualitative research is tied to a theoretical or epistemological premise and this study might be critiqued for a lack of a pre-existing theoretical position for the collection of this qualitative data. However, the absence of an initial theoretical position embedded in the question made participants' narratives of LBP distinctly authentic. The use of thematic analysis and an examination of narrative types provided a flexibility that afforded new insights into the manner in which LBP is experienced and represented. There is an inherent accountability when interpreting other people's stories. The number of stories reviewed in the current study and the similarities among their narratives provides a favourable level of transferability to the themes and analysis of narrative structure.

Conclusion

Although we do not claim that LBP narratives have the intensity of narratives that Frank [18] refers to as those of the 'deeply' ill, identifying the type of narrative in participants' LBP stories as chaos narratives is significant in that it offers an explanation for the disparaging view of LBP in society. Appreciating that LBP narratives are structured as chaos narratives offers the potential for healthcare professionals to recognise the challenges of listening to LBP narratives and to intentionally facilitate constructive interactions that will augment a different experience for patients. The structure of LBP narratives perpetuates LBP as an illness of questionable legitimacy by distancing the sufferer from others and limiting their opportunity to find meaning in the telling and retelling of their LBP story. Hence, both internal and external forces keep them stuck in a chaos narrative. Participants' narratives were about genuine suffering, physical and emotional pain, similar to other illnesses and were worthy of social recognition as a sickness. Yet, despite their desire to enter into an empathetic relationship with others that will lead to a recognition and acknowledgement of their experience, the structure of their narrative negates the likelihood of their needs being fulfilled.

Acknowledgements

This research was supported by a grant from Boston Foundation and Liberty Mutual. Thanks to Dr. E Crepeau, University of New Hampshire for her thoughtful comments that assisted in the preparation of this manuscript. A grant from Boston Foundation and Liberty Mutual supported this research. We thank the study's participants, who gladly and passionately shared their experiences, the clinicians who recruited them, Margo Paterson, Queen's University, Canada, for assistance with coding and data analysis, and Elizabeth Crepeau, Professor Emeritus, UNH, for her guidance in the preparation of this manuscript.

References

1. Kirmayer LJ, Young A. Culture and somatisation: clinical, epidemiological and ethnographic perspectives. *Psychosom Med* 1998;60:420-430.
2. McPhillips-Tangum CA, Cherkin DC, Rhodes LA, Markham C. Reasons for repeated medical visits among patients with chronic back pain. *J Gen Intern Med* 1998;13:289-295.
3. Borkan J, Reis S, Hermoni D, Biderman A. Talking about pain: a patient centred study of low back pain in primary care. *Soc Sci Med* 1995;40:977-988.
4. Tulder MWV, Koes BW. Low back pain. *Am Fam Physician* 2002;65:925-927.
5. Corbett M, Foster N, Ong BN. Living with low back pain: stories of hope and despair. *Soc Sci Med* 2007;65:1584-1594.
6. Osborn M, Smith JA. The person experience of chronic lower back pain: an interpretative phenomenological analysis. *Br J Health Psychol* 1998;3:65-83.
7. Smith JA, Osborn M. Pain as an assault on the self: an interpretative phenomenological analysis of the psychological impact of chronic benign low back pain. *Psychol Health* 2007;22:517-534.
8. Rhodes LA, McPhillips-Tangum CA, Markham C, Klenk R. The power of the visible: the meaning of diagnostic tests in chronic back pain. *Soc Sci Med* 1999;48:1189-1203.
9. Tarasuk V, Eakin JM. Back problems are for life: perceived vulnerability and its application for chronic disability. *J Occup Rehabil* 1994;4:55-64.
10. Johansson E, Hamberg K, Lindgren G, Westman G. 'I've been crying my way' - qualitative analysis of a group of female patients' consultation experience. *Fam Pract* 1996;13:498-503.
11. Johansson E, Lindberg P. Low back pain patients in primary care: subgroups based on the multidimensional pain inventory. *Int J Behav Med* 2000;7:340-352.
12. Lillrank A. Back pain and the resolution of diagnostic uncertainty in illness narratives. *Soc Sci Med* 2003;57:1045-1054.
13. Charmaz K. Stories and silences: disclosures and self in chronic illness. *Qual Inq* 2002;8:302-328.
14. Murray M. Levels of narrative analysis in health psychology. *J Health Psychol* 2000;5:337-347.
15. Frank A. *The Wounded Story Teller: Body, Illness and Ethics*. Chicago: The University of Chicago Press; 1995.
16. Braun V, Clarke V. Using thematic analysis in psychology. *Qual Res Psychol* 2006;3:77-101.
17. Holloway I, Todres L. The status of method: flexibility, consistency and coherence. *Qual Res* 2003;3:345-357.
18. Frank A. Just listening: narrative and deep illness. *Families Syst Health* 1998;16:197-212.
19. Albon M. *Tuesdays with Morrie: An Old Man, A Young Man and Life's Greatest Lesson*. New York: Doubleday; 1997.
20. Little BR. Personal projects and the distributed self: aspects of a conative psychology. *Psychol Perspect Self* 1993;4:157-185.
21. Vroman K, Chamberlain K, Warner R. A personal projects analysis: examining adaptation to low back pain. *J Health Psychol*. Submitted for review.
22. Stein H. 'Sick people' and 'trolls': a contribution to the understanding of the dynamics of physician explanatory models. *Cult Med Psychiatry* 1986;10:221-229.
23. Hadler NM, Carey TS, Garrett J. The influence of indemnification by workers' compensation insurance on recovery from acute backache. *Spine* 2004;20:2710-2715.
24. Dumit J. Illnesses you have to fight to get: facts as forces in uncertain, emergent illness. *Soc Sci Med* 2006;62:577-590.
25. Cambell C, Muncer SJ. The causes of low back pain: a network analysis. *Soc Sci Med* 2005;60:409-419.
26. Asbring P. Chronic illness-a disruption of life: identity-transformation among women with chronic fatigue syndrome and fibromyalgia. *J Adv Nurs* 2001;34:312-319.
27. Ezzy D. Illness narratives: time, hope and HIV. *Soc Sci Med* 2000;50:605-617.
28. Thomas-Maclean R. Understanding breast cancer stories via Frank's narrative types. *Soc Sci Med* 2004;58:1647-1657.
29. Whitehead LC. Quest, Chaos, and restitution: living with chronic fatigue syndrome/myalgic encephalomyelitis. *Soc Sci Med* 2006;62:2236-2245.
30. Hoogendoorn WE, van Poppel M, Bongers P, Koes B, Baxter L. Systematic review of psychosocial factors at work and private life as risk factors for back pain. *Spine* 2000;25:2114-2125.

31. Johansson E, Hamberg K, Westman G, Lindgren G. The meaning of pain: an exploration of women's descriptions of symptoms. *Soc Sci Med* 1999;48:1791–1802.
32. Werner A, Isaksen LW, Malterud K. "I am not the kind of woman who complains of everything": illness stories on self and shame in women with chronic pain. *Soc Sci Med* 2004;59:1035–1045.
33. Hadler NM. Socioeconomics of back pain. *Nat Clin Practice* 2006;2:230–231.
34. Glenton C. Chronic back pain suffers-striving for the sick role. *Soc Sci Med* 2003;57:2234–2252.
35. Kugelman R. Complaining about chronic pain. *Soc Sci Med* 1999;49:1663–1676.
36. McCombe PF, Fairbank JC, Cockersole BC, Pynsent PB. Reproducibility of physical signs in low back pain. *Spine* 1989;14:908–918.
37. Jackson JE. After a while no one believes you. In: Good MJD, Brodwin PE, Good BJ, Kleinman A, editors. *Pain as Human Experience: An Anthropological Perspective*. Berkeley: University of California Press; 1992. pp 138–167.
38. Mattingly C. The concept of therapeutic employment. *Soc Sci Med* 1994;38:811–822.
39. Borkan J, Tulder MV, Reis S, Scheone ML, Croft P, Hermoni D. Advances in the field of low back pain in primary care: a report from the fourth international forum. *Spine* 2002;27:E128–E132.